NEW YORK STATE

10 NYCRR 405.4

Hospital Sepsis Screening and Treatment Protocols

Enacted pursuant to the Rory Staunton Foundation advocacy, this regulation establishes mandatory sepsis screening protocols for all hospitals licensed in New York State. Adopted 2013; amended 2020.

Effective Date: December 31, 2013

Authority: Public Health Law §§ 2803, 2803-l, 2805-k

PREAMBLE

In memory of Rory Staunton, age 12, who died of severe sepsis on April 1, 2012, after being discharged from a New York City emergency department, this regulation requires hospitals to implement evidence-based protocols for the early identification and treatment of sepsis in all patient populations, including pediatric patients.

The sections that follow establish screening requirements, escalation pathways, documentation standards, and re-presentation protocols applicable to all patients presenting to emergency departments, urgent care facilities, and inpatient units.

§ 405.4.1 General Screening Requirements

Every pediatric patient presenting to the emergency department shall be screened for sepsis using age-appropriate vital sign thresholds and clinical criteria. Screening shall occur within 15 minutes of triage arrival and shall be documented in the patient's chart prior to any disposition decision.

Screening protocols shall incorporate the following elements at minimum: (a) heart rate; (b) respiratory rate; (c) temperature; (d) mental status assessment; (e) documentation of any recent wound, injury, or suspected source of infection; and (f) comparison of vital signs against age-adjusted reference ranges.

§ 405.4.2 Pediatric-Specific Thresholds

Pediatric-specific vital sign thresholds shall be applied per PALS reference ranges, with age-adjusted criteria for tachycardia, tachypnea, and hypotension. Adult sepsis screening thresholds shall not be applied to patients under 18 years of age.

Each hospital shall maintain a written pediatric sepsis screening policy that references published age-band vital sign criteria (e.g., PALS 2020, Pediatric Surviving Sepsis Campaign guidelines) and shall train all triage nursing staff in the application of those criteria.

§ 405.4.3 Re-Presentation Clause

Patients re-presenting to the emergency department within 72 hours of discharge shall receive heightened screening and mandatory physician re-evaluation prior to any subsequent discharge.

This section was added specifically in response to the 2012 Staunton case, in which the decedent was discharged from two consecutive emergency department visits before his sepsis was identified. No patient who has been discharged and returns with worsening symptoms within the 72-hour window may be discharged a second time without direct attending physician evaluation, documentation of the clinical reasoning, and re-screening using the full sepsis protocol.

Patients demonstrating accelerating trajectory markers - including rising heart rate, falling blood pressure, altered mental status, or progression of localized findings - shall be considered high-suspicion for sepsis regardless of current threshold status.

§ 405.4.4 Documentation Requirements

All sepsis screening results shall be documented in the patient's medical record. Required documentation elements include: the screening tool used; the specific vital sign values against age-adjusted thresholds; the presence or absence of suspected infection source; mental status assessment; and the disposition decision with clinical justification.

§ 405.4.5 Escalation Pathway

Any patient triggering the sepsis screen shall not be discharged without direct physician evaluation and documentation of the reasoning for discharge.

When the sepsis screen is positive, the following protocol applies: (a) attending physician shall be notified within 5 minutes; (b) the 1-hour sepsis treatment bundle shall be initiated as clinically appropriate; (c) serum lactate and blood cultures shall be obtained before antibiotic administration; (d) empiric broad-spectrum antibiotics shall be administered within 60 minutes of identification; and (e) the patient shall not be discharged without reassessment.

Violations of this section shall be reported to the New York State Department of Health and may result in enforcement action under 10 NYCRR Part 405.